Case study: Amira

Condition: **Cerebral palsy – spastic quadriplegia (GMFCS V)**

4-year-old girl · Paediatric community occupational therapy home visit

Amira Hassan is a 4-year-old girl with **spastic quadriplegic cerebral palsy** who lives with her parents, Leila and Omar, and her 8-year-old brother, Yusuf, in a townhouse in Logan. Amira is classified at GMFCS level V and MACS level V. She does not use words, communicating through facial expression, vocal sounds and crying, and she is fed entirely through a gastrostomy. She takes regular medication for muscle tone and seizures. Her supports are funded through her NDIS plan.

Amira's physiotherapy notes record that her hip surveillance X-ray in June showed stable hips, and the dietitian's note from last month documents steady weight gain on her current feeds. Leila tells you that Amira has had "no seizures for ages" and that the family is keen to finalise a new wheelchair before the NDIS plan review deadline on Friday.

You have been asked to complete a seating and postural review at home so that a quote for a new tilt-in-space wheelchair can be submitted. Amira has outgrown the wheelchair she has used since she was two, and she now slumps to the right in it. A supplier's representative has brought a trial chair. When you arrive, Amira is lying on a mat in the lounge room watching cartoons, and she smiles when Leila sings to her.

When Leila lifts Amira onto the change table before the trial, Amira lets out a high-pitched scream as her left leg is straightened, and she keeps crying for several minutes. Leila looks embarrassed. "She's just grumpy. She's getting her back teeth," she says. "I've been giving her Panadol through her tube since yesterday." She adds that Amira has been "really cranky for two days" and has not been sleeping well.

As you help position Amira in the trial chair, you notice that her left thigh looks fuller than her right and feels warmer to touch. Amira cries again when her left knee is bent and holds that leg stiffly against her body. You cannot see any bruising. When you gently ask whether anything happened, Leila says quickly, "No, nothing. She didn't fall. Nobody dropped her." Omar, who has been watching quietly from the kitchen, speaks to Leila in Arabic. She answers him briefly and tells you, "He just says she's tired." You are not sure how much of the conversation Omar has understood, and the referral does not say whether an interpreter is needed for either parent.

Leila then asks whether the new chair could have a tray for a communication device, and when Amira could start toilet training. She also mentions that a support worker helped bath Amira earlier in the week, but she cannot remember which day, and that Amira "cried the whole time", which is unusual because she normally loves her bath.

There is no clear documentation regarding Amira's bone health, whether she has had any previous injuries, which carers have handled her over the past week, or when she was last reviewed by her paediatrician. The referral does not mention any recent illness.

The supplier's representative checks his watch and says he needs the measurements within ten minutes because he has another trial across town. Leila glances at the clock and says she must leave at 2.45 pm to collect Yusuf from school. "Can we just do the chair quickly?" she asks. "The plan review is Friday and I don't want to lose the funding."

You are the occupational therapy student completing a seating review with Amira and her family, and your clinical supervisor is not currently in the room.

Present Amira's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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